MEDIBANK

Healthcare Intelligence Briefing

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Polypharmacy in Australia in 2025	AIHW	26 Aug 2026	19 Oct 2026
Alcohol, tobacco & other drugs in Australia	AIHW	1 Dec 2025	19 Oct 2026
Illicit drug use	AIHW	7 Oct 2026	19 Oct 2026
Diabetes: Australian facts	AIHW	13 May 2026	19 Oct 2026
General practice care for people with a mental health condition	AIHW	23 Oct 2026	24 Oct 2026
E-scooter and e-bike injuries and deaths in Australia: national trends and data limitations	AIHW	15 Sep 2026	24 Oct 2026

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Executive Summary

Cancer spending has more than doubled in a decade to $21.7 billion in 2024–25, a sustained rise that may continue to drive hospital, pharmaceutical and referred-services claims. Fall injuries are now the single costliest specific condition nationally at $5.7 billion, overtaking osteoarthritis, with much of that spending flowing through hospital admissions. Non-government spending on private hospitals fell 1.6% after inflation while public hospital spending surged well above its decade average — a widening divergence that may affect the value proposition of hospital cover. Musculoskeletal disorders dominate private hospital admissions at $5.8 billion, the highest of any disease group, representing a concentrated and current claims exposure. Referred medical services grew 4.2% after inflation, the fastest of any major spending category, which may add pressure to specialist gap payments and ancillary benefits.

Three early signals are worth watching. The National Lung Cancer Screening Program, launched July 2025, is not yet costed but could generate downstream private-sector activity if screen-detected cancers lead to earlier surgical and oncology treatment. Hallucinogen use among people in their 20s more than doubled between 2016 and 2022–23, a trend that may eventually increase emergency and mental-health service demand among younger members. The ACT experience suggests tighter e-scooter regulation can reverse hospitalisation trends, a pattern worth monitoring if larger states introduce similar rules.

Early signals based on Insights 33, 34, 35

Top 5 Insights

1	**Cancer Spending Decade Growth**	73% real increase in cancer spending since 2013–14	Insight 4 →
2	**Fall Injuries Top Spending Condition**	$5.7 billion — highest spending for any specific condition in 2024–25	Insight 5 →
3	**Private Hospital Activity Contracting**	non-government spending on private hospitals fell 1.6% in real terms	Insight 1 →
4	**Musculoskeletal Private Hospital Dominance**	$5.8 billion — highest spending disease group in private hospitals	Insight 6 →
5	**Referred Medical Services Acceleration**	4.2% real increase in 2024–25, fastest of any major spending category	Insight 3 →

Clinical Insights

HIGH PRIORITY *Insight 1 · AIHW · Clinical*

Private hospital activity and non-government spending are both contracting: non-government spending on private hospitals fell 1.6% after inflation in 2024–25 while separations in private hospitals also declined slightly by 0.3%, contrasting with a 5.2% real increase in public hospital spending.

Evidence

"From 2023–24 to 2024–25, the Australian Government spending on private hospitals increased by $0.1 billion (1.2%) in real terms. Non-government spending on private hospitals decreased by $0.2 billion (1.6%) in real terms. During the same period, the number of separations in private hospitals decreased slightly by 0.3% (AIHW 2026)."

Section: Private hospitals · Source: Health expenditure Australia 2024–25

Why it matters

This directly affects Medibank's core hospital product: non-government spending on private hospitals fell 1.6% in real terms while private hospital separations also declined 0.3%, even as public hospital activity grew strongly. The divergence suggests members may be shifting care to the public system, which could erode the value proposition of hospital cover. Rated high because it points at the heart of Medibank's hospital claims and product relevance. Worth checking whether Medibank's own private hospital claims mirror this contraction.

HIGH PRIORITY *Insight 2 · AIHW · Clinical*

Public hospital spending surged 5.2% after inflation in 2024–25 to $99.9 billion, above the decade average of 4.6%, driven mainly by a 2.3% rise in public hospital separations — the fastest growth in the post-pandemic period.

Evidence

"Spending on public hospital services was estimated at $99.9 billion in 2024–25, compared with $94.9 billion in 2023–24 (Figure 25). This represented a real increase of 5.2%, above the decade average annual growth rate of 4.6%. During the same time, the number of separations in public hospitals increased by 2.3% (AIHW 2026)."

Section: Figure 25: Public hospital services spending, by source of funds, constant prices (a), 2014–15 to 2024–25 · Source: Health expenditure Australia 2024–25

Why it matters

Public hospital spending surging 5.2% in real terms — above the decade average — alongside the fastest post-pandemic growth in public separations may indicate growing demand that could spill over into private hospital waiting times, or alternatively draw insured patients toward the public system. Combined with the private hospital contraction in INSIGHT_001, this widens the public-private divergence that directly affects Medibank's hospital cover value. Rated high because the sustained above-trend growth signals a structural demand shift worth investigating in Medibank's own hospital claims and member behaviour data.

HIGH PRIORITY *Insight 3 · AIHW · Clinical*

Referred medical services spending grew 4.2% after inflation in 2024–25 to $26.3 billion, the fastest growth of any major spending category, with non-government spending in this area growing at 4.0% per year on average over the decade.

Evidence

"In 2024–25, spending on referred medical services increased by 4.2% ($1.1 billion) in real terms. Growth was driven by higher Australian Government spending, which increased by 4.6% ($0.8 billion), and higher spending from non-government sources, which rose by 3.2% ($0.3 billion). Over the decade to 2024–25, spending on referred medical services grew by an average of 1.2% per year in real terms. This growth was driven mainly by non-government spending, which increased by an average of 4.0% a year."

Section: Referred medical services · Source: Health expenditure Australia 2024–25

Why it matters

Referred medical services — specialist consultations, pathology, imaging and allied health — grew 4.2% in real terms, the fastest of any major category, with non-government spending in this area averaging 4.0% annual growth over the decade. This directly affects Medibank through specialist gap payments, ancillary benefits and member out-of-pocket costs for referred services. Rated high because the sustained above-average growth in a category where private insurers and individuals bear significant costs represents a material and ongoing claims pressure. Worth examining whether Medibank's specialist and referred-service claims show a similar acceleration.

HIGH PRIORITY *Insight 4 · AIHW · Clinical*

Cancer spending has more than doubled in a decade and now dominates all disease groups: $21.7 billion in 2024–25, up from $9.1 billion in 2013–14, a 73% real increase, making it the highest-spending group for eight consecutive years.

Evidence

"For the past eight years (since 2017–18), spending on cancers and other neoplasms was higher than spending for any other burden of disease group and has more than doubled from $9.1 billion in 2013–14 to $21.7 billion in 2024–25. In real terms (after removing the effects of inflation) spending on cancer has increased 73% since 2013–14."

Section: Figure 13: Ranking of spending on disease groups(a), by sex, current and constant prices, 2013–14 to 2024–25 · Source: Health system spending on disease and injury in Australia 2024–25

Why it matters

Cancer is the highest-spending disease group nationally at $21.7 billion, with a 73% real increase over a decade, which could directly affect Medibank's hospital, pharmaceutical and referred-services claims for members with cancer. The sustained eight-year dominance and steep growth trajectory suggest ongoing pressure on private hospital admissions, oncology treatments and high-cost medicines. It is rated high because the scale, trend and direct claims connection are all strongly established. Worth examining whether Medibank's own cancer-related claims mirror this national growth rate and spending concentration.

HIGH PRIORITY *Insight 5 · AIHW · Clinical*

Injuries from falls are now the single highest-spending specific condition at $5.7 billion in 2024–25, overtaking osteoarthritis, which held the top position from 2013–14 to 2018–19.

Evidence

"Across all areas of expenditure combined, from 2022–23 to 2024–25, the highest spending was for injuries from falls. Between 2013–14 and 2018–19 the highest spending was for osteoarthritis."
Section: Figure 16: Ranking of spending on diseases, by area of expenditure, current and constant prices, 2013–14 to 2024–25 · Source: Health system spending on disease and injury in Australia 2024–25

Why it matters

Fall injuries at $5.7 billion are now the single costliest specific condition nationally, overtaking osteoarthritis, and much of this spending flows through hospital admissions where private insurers bear significant costs, particularly for older members. Falls drive orthopaedic surgery, rehabilitation and extended hospital stays that are common private-hospital services. It is rated high because the condition directly generates private hospital claims and the spending trajectory shows a sustained shift to the top position since 2022–23. Worth checking whether Medibank's hip and limb fracture claims in older members show a comparable rise.

HIGH PRIORITY *Insight 6 · AIHW · Clinical*

Musculoskeletal disorders dominate private hospital admissions ($5.8 billion, the highest of any disease group), while cardiovascular diseases lead public hospital admissions ($8.3 billion), indicating a clear split in where each condition is treated.

Evidence

"For public hospital admitted patients, the highest spending was for cardiovascular diseases ($8.3 billion), followed by cancer and other neoplasms ($7.7 billion) and gastrointestinal disorders ($7.0 billion). For private hospital admitted patients, musculoskeletal disorders had the highest spending ($5.8 billion), followed by cardiovascular diseases ($3.1 billion) and cancer and other neoplasms ($3.0 billion)."
Section: Figure 5: Expenditure for Burden of Disease groups(a) by area of expenditure, 2024–25 · Source: Health system spending on disease and injury in Australia 2024–25

Why it matters

Musculoskeletal disorders are the highest-spending disease group in private hospitals at $5.8 billion, nearly double the next group, which maps directly onto Medibank's hospital claims for joint replacements, spinal procedures and related orthopaedic care. The clear public–private split shows that private insurers bear a disproportionate share of musculoskeletal treatment costs compared with other disease groups. It is rated high because the spending is already concentrated in the private system and represents a current material claims exposure. Worth benchmarking Medibank's musculoskeletal hospital claims against this national private-hospital share.

HIGH PRIORITY *Insight 7 · AIHW · Clinical*

Rising cost per service — not more people using services — was the second-largest driver of health spending growth over the decade, adding $14.0 billion (33.6%) after inflation, and it was concentrated in public hospitals, emergency departments, and outpatient and specialist services.

Evidence

"Higher average expenditure per service increased spending for public hospital admitted patients, emergency department and outpatient services, and across general practitioner, allied health and specialist services. In all other areas, this factor reduced spending."
Section: Decomposition of disease expenditure by detailed area of expenditure · Source: Health system spending on disease and injury in Australia 2024–25: Growth decomposition of health system spending on disease and injury in Australia

Caveat Results represent estimated contributions under the decomposition model and should not be interpreted as independent causal effects.

Why it matters Rising cost per service added $14.0 billion in real spending over the decade, concentrated in public hospitals, emergency departments, outpatient and specialist services — areas that also affect private hospital pricing benchmarks and specialist fees paid through private cover. This signals that unit-cost inflation, not volume growth, is a dominant spending pressure, which could flow through to Medibank's hospital contracts and specialist gap payments. It is rated high because the magnitude is large, the trend spans a full decade, and the pattern points to specific service areas Medibank can examine. Worth checking whether Medibank's own per-service costs in hospital and specialist claims mirror this public-sector pattern.

HIGH PRIORITY *Insight 8 · AIHW · Clinical*

For coronary heart disease, falling disease burden reduced spending by $971.8 million over the decade, but higher expenditure per service in public hospital admissions alone added $511.4 million, largely offsetting those gains.

Evidence "For coronary heart disease, changes in disease burden drove an overall decrease in expenditure of –$971.8 million, whilst services per DALY and expenditure per service drove an increase in expenditure of $316.0 million and $232.1 million respectively. Furthermore, the largest contributor to change for coronary heart disease across all areas of expenditure was expenditure per service within public hospital admitted patients which drove an increase of $511.4 million between 2015–16 and 2024–25."
Section: Decomposition of disease expenditure by condition and detailed area of expenditure · Source: Health system spending on disease and injury in Australia 2024–25: Growth decomposition of health system spending on disease and injury in Australia

Caveat Results represent estimated contributions under the decomposition model and should not be interpreted as independent causal effects.

Why it matters Coronary heart disease is a major private hospital admission category, and the finding that falling disease burden saved nearly $1 billion yet rising cost per public hospital admission added $511 million suggests unit-cost inflation is eroding clinical gains. If a similar dynamic applies in private hospitals, Medibank could face rising per-episode cardiac costs even as the number of cardiac patients declines. It is rated high because the dollar magnitudes are substantial and the condition is core to hospital cover. Worth investigating whether Medibank's cardiac admission costs per episode show the same upward pressure.

HIGH PRIORITY *Insight 9 · AIHW · Clinical*

For osteoarthritis, higher expenditure per service drove a $241.1 million increase in public hospital admissions but simultaneously reduced private hospital admitted patient spending by $395.4 million — suggesting a shift in where osteoarthritis care is being delivered or priced.

Evidence "For osteoarthritis, changes in expenditure per service were associated with an increase of $241.1 million within public hospital admitted patient services whilst driving a decrease in private hospital admitted patient services and PBS expenditure of -$395.4 million and -$85.8 million respectively."
Section: Decomposition of disease expenditure by condition and detailed area of expenditure · Source: Health system spending on disease and injury in Australia 2024–25: Growth decomposition of health system spending on disease and injury in Australia

Caveat Results represent estimated contributions under the decomposition model and should not be interpreted as independent causal effects.

Why it matters The $395 million reduction in private hospital osteoarthritis spending driven by cost-per-service changes, alongside a $241 million increase in public hospitals, may signal a shift in where joint replacement and related procedures are being delivered or how they are priced. This could directly affect Medibank's hospital claims volume and contract negotiations for orthopaedic procedures. It is rated high because the dollar shift is large, osteoarthritis surgery is a high-volume private hospital category, and the pattern spans a decade. Worth examining whether Medibank's own orthopaedic claims show a corresponding public-private migration.

HIGH PRIORITY *Insight 10 · AIHW · Clinical*

For lower respiratory infections (including influenza and pneumonia), service use per DALY surged to add $1.1 billion in spending even as falling disease burden reduced costs by $979.8 million — meaning more services are being delivered per unit of illness.

Evidence "For lower respiratory infections (including influenza and pneumonia), the largest overall driver of change was services per DALY, which was associated with an increase in expenditure of $1.1 billion. Conversely, changing disease burden drove a decrease in overall expenditure of -$979.7 million, highlighting the changing dynamic between disease burden and service use across the period."
Section: Decomposition of disease expenditure by condition and detailed area of expenditure · Source: Health system spending on disease and injury in Australia 2024–25: Growth decomposition of health system spending on disease and injury in Australia

Caveat Results represent estimated contributions under the decomposition model and should not be interpreted as independent causal effects. The service use per DALY factor reflects changes relative to disease burden, not necessarily that fewer or more services were provided in absolute terms.

Why it matters Lower respiratory infections saw $1.1 billion in additional spending from more services per unit of illness, even as disease burden fell by nearly $1 billion — meaning the health system is delivering substantially more care per case. This could affect Medibank through higher per-episode hospital and GP claims for respiratory conditions among members, particularly older cohorts. It is rated high because the spending shift is very large and the pattern suggests a structural change in how respiratory illness is managed. Worth checking whether Medibank's respiratory-related claims per episode have risen similarly.

HIGH PRIORITY *Insight 11 · AIHW · Clinical*

Deaths involving heroin have shifted markedly: heroin's share of opioid-induced deaths rose from 22% (237 deaths) in 2014 to 49% (501 deaths) in 2024, while pharmaceutical opioid deaths fell from 83% to 62% over the same period.

Evidence "In 2024, all opioids excluding heroin were involved in 62% (635 deaths) of opioid-induced deaths, down from 83% (917 deaths) in 2014. Heroin was involved in 49% of opioid-induced deaths in 2024 (501 deaths), up from 22% (237 deaths) in 2014 (Table NMD2)."
Section: Figure 2: Drug-induced deaths for all opioids excluding heroin and heroin only, 1997 to 2024 · Source: Alcohol, tobacco & other drugs in Australia

Caveat Deaths registered in 2023 and 2024 are based on preliminary data and are subject to further revision by the ABS.

Why it matters This may affect Medibank through hospital admissions, emergency presentations and treatment claims related to opioid use among males aged 35–54, a group likely to hold private cover. Heroin-induced deaths more than doubling from 237 to 501 over a decade signals a major shift in the opioid harm profile, which could change the mix of overdose presentations, rehabilitation services and opioid pharmacotherapy demand members require. It is rated high because the trend is sustained over ten years, the magnitude of change is large, and it points to a concrete shift in treatment needs. Worth checking whether Medibank claims data show a corresponding rise in heroin-related hospitalisations or a change in opioid substitution therapy utilisation.

HIGH PRIORITY *Insight 12 · AIHW · Clinical*

GHB hospitalisations have risen sharply and continuously, reaching 14 per 100,000 in 2023–24 — nearly six times the rate recorded in 2015–16 — the highest level since monitoring began.

Evidence

"gamma-hydroxybutyrate (GHB) rose from 2.4 to 14 hospitalisations per 100,000 in 2023–24, the highest level since monitoring began"
Section: Figure 2: Hospitalisations with a drug-related principal diagnosis (excluding alcohol), by drug type, 2015–16 to 2023–24 · Source: Illicit drug use

Why it matters

This may directly affect Medibank's hospital claims for drug-related admissions, as GHB hospitalisations have nearly six-folded over eight years to their highest recorded level. The sustained upward trend signals growing acute-care demand that could be reflected in emergency and inpatient costs for privately insured members. It is rated high because the magnitude of change is large, the trend is continuous across the full nine-year series, and the hospitalisation data provide a concrete utilisation anchor. Worth checking whether Medibank's own drug-related hospital claims show a comparable GHB-driven increase.

HIGH PRIORITY *Insight 13 · AIHW · Clinical*

Methamphetamine hospitalisations peaked at 56 per 100,000 in 2023–24, the highest rate in the nine-year series, with over 15,100 hospital stays — making it the single largest illicit drug driver of inpatient admissions.

Evidence

"methamphetamine fluctuated, rising to a peak of 56 hospitalisations per 100,000 in 2023–24"
Section: Figure 2: Hospitalisations with a drug-related principal diagnosis (excluding alcohol), by drug type, 2015–16 to 2023–24 · Source: Illicit drug use

Why it matters

Methamphetamine is now the single largest illicit-drug driver of inpatient admissions nationally, peaking at 56 per 100,000 with over 15,100 hospital stays in 2023–24, which could translate into substantial hospital and mental-health claims for Medibank members. The clinical burden is severe, encompassing acute toxicity, psychosis and longer-stay psychiatric admissions. It is rated high because the volume is large and the rate has reached a new peak, even though the trend has fluctuated rather than risen steadily. Medibank could examine its own claims to quantify methamphetamine-related hospital and rehabilitation costs.

HIGH PRIORITY *Insight 14 · AIHW · Clinical*

People with schizophrenia or affective psychosis have nearly double the rate of type 2 diabetes compared with people without mental health conditions (13% versus 6.7%), pointing to a high-cost comorbidity burden in this group.

Evidence

"The proportion of type 2 diabetes was almost double among people with schizophrenia and/or affective psychosis (13%) compared with people with anxiety, depression, and/or phobia/compulsive disorders (6.9%) and people without these conditions (6.7%)."
Section: Figure 5: Types of physical health conditions among people with a recorded mental health condition in general practice in WA, 2023–24 · Source: General practice care for people with a mental health condition

Caveat

Findings are descriptive; no causal inference is possible. Data limited to WA and exclude ACCHOs.

Why it matters This may affect Medibank members with schizophrenia or affective psychosis who face nearly double the rate of type 2 diabetes (13% vs 6.7%), driving higher GP visits, medication use, specialist referrals and potential hospital admissions for diabetes complications. The comorbidity burden creates a concrete pathway through both healthcare costs and member health outcomes. It is rated high because the clinical significance of this gap is substantial and identifies a specific group where integrated physical-mental health care could reduce downstream costs. Worth checking whether Medibank claims for members with psychosis-related diagnoses show elevated diabetes-related service use.

HIGH PRIORITY *Insight 15 · AIHW · Clinical*

Obesity is recorded in 38% of people with schizophrenia or affective psychosis, nearly double the 21% rate in people without mental health conditions, compounding cardiovascular and metabolic risk in this cohort.

Evidence "The next most common biomedical risk factor was obesity. The proportion of recorded obesity was almost double for people with schizophrenia and/or affective psychosis (38%), in comparison with people without these conditions (21%)."
Section: Figure 6: Proportions of biomedical risk factors among people with a recorded mental health condition in general practice in WA, 2023–24 · Source: General practice care for people with a mental health condition

Caveat Obesity recording completeness is noted as 70–80% in the technical notes, so the true rate may be higher. Findings are descriptive only.

Why it matters Obesity at 38% in people with schizophrenia or affective psychosis — nearly double the rate in those without mental health conditions — compounds cardiovascular and metabolic risk and could drive higher claims for related hospitalisations, surgeries and chronic disease management among affected Medibank members. The link is direct: antipsychotic-associated weight gain and lifestyle factors create a well-recognised pathway to costly physical health complications. It is rated high because the gap is large and points to a specific member subgroup where targeted intervention could improve outcomes. Medibank could examine whether its members on antipsychotic medications show elevated obesity-related service use.

HIGH PRIORITY *Insight 16 · AIHW · Clinical*

Up to 91% of people with co-occurring anxiety/depression and psychosis have at least one recorded biomedical risk factor or health behaviour, compared with 66% of people without mental health conditions, confirming a very high physical health burden in this group.

Evidence "The proportion of people who had one or more recorded biomedical risk factor and/or health behaviour was: - 78% for people with anxiety, depression, and/or phobia/compulsive disorders - 87% for people with schizophrenia and/or affective psychosis - 91% for people with anxiety, depression, and/or phobia/compulsive disorders as well as schizophrenia and/or affective psychosis - 66% for people without these conditions."
Section: Biomedical risk factors · Source: General practice care for people with a mental health condition

Caveat Risk factor recording completeness varies (70–80% for smoking and alcohol); true rates may be higher. Findings are descriptive only.

Why it matters With up to 91% of people with co-occurring anxiety/depression and psychosis having at least one biomedical risk factor, this points to a very high physical health burden that could drive substantial healthcare use and costs for affected Medibank members across multiple service types — GP management, specialist referrals, hospitalisations and chronic disease programs. The near-universal presence of modifiable risk factors in this group creates a clear pathway for targeted prevention and care coordination. It is rated high because the sheer concentration of risk factors identifies a specific member cohort where integrated care models could materially improve outcomes and reduce downstream costs. Worth investigating whether Medibank members with multiple mental health diagnoses show disproportionate physical health service use.

HIGH PRIORITY *Insight 17 · AIHW · Clinical*

Powered scooter fall hospitalisations have nearly tripled since 2019–20, reaching 15 per 100,000 population in 2024–25, with 4,089 hospital admissions that year — a sustained, rapid rise that shows no sign of plateauing.

Evidence "Since 2019–20, hospitalisations from falls involving powered scooters have nearly tripled, reaching 15 per 100,000 population in 2024–25."
Section: Trends in hospitalisations for transport groups that include e-scooters and e-bikes · Source: E-scooter and e-bike injuries and deaths in Australia: national trends and data limitations

Caveat Current ICD-10-AM codes cannot separately identify e-scooter injuries from other powered scooter injuries, including mobility scooters used by older people, so the true e-scooter-specific rate may differ.

Why it matters A near-tripling of powered scooter fall hospitalisations to over 4,000 admissions in 2024–25 could be driving a growing volume of trauma-related hospital claims for Medibank members, including emergency presentations, orthopaedic procedures and rehabilitation. The sustained upward trend with no sign of plateauing suggests this demand pressure will continue. It is rated high because the scale is already large and directly linked to hospital use, though current coding cannot separate e-scooter injuries from mobility scooter injuries. Worth checking whether Medibank hospital claims for powered scooter fall injuries mirror this national trajectory.

HIGH PRIORITY *Insight 18 · AIHW · Clinical*

Boys aged 12–15 years are the highest-risk group by far: their hospitalisation rate for powered scooter falls rose 15-fold from 3.6 to 54 per 100,000 between 2019–20 and 2024–25, more than four times the rate for girls the same age.

Evidence "The increase was largest among boys aged 12–15 years, with rates rising 15-fold from 3.6 to 54 per 100,000. Among girls, rates increased 5-fold, from 2.5 to 12 per 100,000."
Section: Hospitalisations for the transport group that includes falls involving powered scooters · Source: E-scooter and e-bike injuries and deaths in Australia: national trends and data limitations

Caveat Most jurisdictions do not permit public e-scooter use under 16, so this age group's exposure partly reflects jurisdictions with lower age limits (Queensland, ACT) and supervised use provisions.

Why it matters A 15-fold rise in hospitalisation rates among boys aged 12–15 may affect Medibank families policies, since these children are typically covered as dependants and their injuries could generate emergency, surgical and rehabilitation claims. The concentration in a narrow age-sex group makes the pattern distinctive and potentially identifiable in Medibank data. It is rated high because the magnitude is extreme and the trend is sustained, even though coding limitations mean some injuries may involve mobility scooters rather than e-scooters. Worth investigating whether paediatric trauma claims among dependants show a similar spike in relevant age bands.

MEDIUM PRIORITY *Insight 19* · *AIHW* · *Clinical*

Private health insurers spent $1,779 per person covered by a hospital policy in 2024–25, a real decrease of $8 (0.4%) from the prior year, but there is wide state variation — South Australia ($1,994) and Queensland ($1,909) are well above the Northern Territory ($1,058).

Evidence

"In 2024–25, private health insurance providers spent an estimated average of $1,779 per person covered by a private hospital insurance policy. This was a decrease of $8 (0.4%) from 2023–24 in real terms. The average annual growth for the decade to 2024–25 was 1.0% (Figure 20). South Australia ($1,994), Queensland ($1,909), and Tasmania ($1,858) had the highest spending by private health insurers per person covered, at more than 1.8 times the amount of the Northern Territory ($1,058) (Figure 20)."

Section: Figure 20: Average per person (a) spending by private health insurance providers for each state and territory, constant prices (b), 2014–15 to 2024–25 · Source: Health expenditure Australia 2024–25

Why it matters

The $1,779 per-person insurer spend and the wide state variation — South Australia nearly double the Northern Territory — could affect how Medibank prices products and manages benefits across regions. The slight real decrease nationally suggests stable or softening claims per member, but the geographic spread may signal differing utilisation patterns or provider cost structures. Rated medium because the overall change is small, but the state-level variation offers a concrete investigation opportunity for Medibank's pricing and network teams.

MEDIUM PRIORITY *Insight 20* · *AIHW* · *Clinical*

Non-government sources have overtaken the Australian Government as the largest funder of primary health care for the first time in the decade, contributing 43.4% ($40.4 billion) versus the Australian Government's 42.7% ($39.7 billion) in 2024–25.

Evidence

"In 2024–25, $93.0 billion was spent on primary health care including public health. Of this, non-government sources contributed $40.4 billion (43.4%), the Australian Government $39.7 billion (42.7%), and state and territory governments $12.9 billion (13.9%) (Figure 27). This was the first time in the decade up to 2024–25, non-government sources accounted for the largest share of primary health care spending."

Section: Figure 27: Primary health care expenditure, by source of funds, constant prices (a), 2014–15 to 2024–25 · Source: Health expenditure Australia 2024–25

Why it matters

Non-government sources overtaking the Australian Government as the largest funder of primary health care for the first time could signal rising out-of-pocket costs and growing demand for ancillary/extras cover among Medibank members. The $40.4 billion non-government share includes individual and insurer contributions, so a shift here may affect how members use and value their extras policies. Rated medium because the structural funding shift is meaningful but the direct private insurance component within non-government spending is not separately quantified.

MEDIUM PRIORITY *Insight 21* · *AIHW* · *Clinical*

Individuals' out-of-pocket spending on health rose 1.8% after inflation to $46.4 billion in 2024–25, with the largest single component being non-PBS medications ($16.6 billion, 35.8% of individual spending), followed by dental ($8.6 billion) and medical services ($7.9 billion).

Evidence

"Individuals spent an estimate of $46.4 billion out-of-pocket on health goods and services in 2024–25. This was a real increase of 1.8% from 2023–24 (Table 20). In 2024–25, individuals spent an estimated $16.6 billion (35.8%) on medications not subsidised through the PBS, including over-the-counter medicines, vitamins and health-related products. They also spent $8.6 billion (18.5%) on dental services and $7.9 billion (17.0%) on both referred and unreferred medical services (Table 20)."
Section: Individual spending · Source: Health expenditure Australia 2024–25

Why it matters

Members' out-of-pocket costs reached $46.4 billion, with dental ($8.6 billion) and medical services ($7.9 billion) being areas where private health insurance extras and gap cover are directly relevant. Rising out-of-pocket spending could increase member expectations of their Medibank policies and influence product satisfaction and retention. Rated medium because the 1.8% real growth is modest in a single year, but the absolute scale and the breakdown across dental and medical services offer useful context for benefits design and member affordability monitoring.

MEDIUM PRIORITY *Insight 22* · *AIHW* · *Clinical*

An estimated $5.5 billion of NDIS spending in 2024–25 was attributable to health-related services — including allied health, nursing, and assistive technology — but this spending is entirely excluded from the national health accounts.

Evidence

"Health spending by participants in the National Disability Insurance Scheme (NDIS) is not currently included in the ANHA. According to the National Disability Insurance Agency (NDIA), $5.5 billion of NDIS spending in 2024–25 was attributable to health-related services (NDIA 2026), in particular, supports for allied health and therapies, nursing and assistive technology. Spending that meets the AIHW definition of health expenditure but funded by the NDIS are not yet included in the estimates presented in this report."
Section: Spending trends by source · Source: Health expenditure Australia 2024–25

Caveat

NDIS health-related spending is excluded from the ANHA estimates; the $5.5 billion figure is sourced from the NDIA and not independently verified within this report.

Why it matters

The $5.5 billion in NDIS health-related spending on allied health, nursing and assistive technology sits outside the national health accounts but could affect Medibank members who hold both NDIS plans and private cover. Overlap or substitution between NDIS-funded and privately insured allied health services may influence extras claims patterns. Rated medium because the scale is substantial and the allied health overlap is plausible, but the data is excluded from the main accounts and the direct private insurance interaction is not quantified. Worth monitoring for potential claims substitution effects.

MEDIUM PRIORITY *Insight 23* · *AIHW* · *Clinical*

Health prices rose 3.89% in 2024–25, outpacing general inflation of 3.3%, producing excess health inflation of 0.57% — a reversal from the negative excess health inflation recorded in 2022–23.

Evidence

"Between 2023–24 and 2024–25, health prices grew by 3.89%. General inflation, measured using the implicit price deflator (IPD) for gross national expenditure (GNE), was 3.3%. As such, 'excess health inflation' was 0.57%, indicating that prices of health goods and services rose slightly faster than prices in the broader economy (Figure 5)."
Section: Figure 5: Annual health inflation (a) and general inflation (b) rates, 2014–15 to 2024–25 · Source: Health expenditure Australia 2024–25

Why it matters

Health prices rising faster than general inflation at 3.89% versus 3.3% could put upward pressure on Medibank's claims costs and ultimately on premiums. The return of positive excess health inflation after a negative period in 2022–23 signals that the cost environment for health services is tightening again. Rated medium rather than high because the 0.57% excess is relatively modest in a single year and it is unclear whether this will persist, but it is a useful input for Medibank's premium-setting and cost-management planning.

MEDIUM PRIORITY *Insight 24* · *AIHW* · *Clinical*

The Australian Government's share of total health spending fell to its lowest point in the decade at 39.2%, while state and territory governments reached their highest share at 30.8%, reflecting a structural shift in the funding balance.

Evidence

"During 2024–25, Australia spent $290.7 billion on health. Government funded more than two-thirds (70.0% or $203.5 billion), comprising 39.2% from the Australian Government and 30.8% from state and territory governments. The remaining 30.0% ($87.1 billion) was funded by non-government sources (Figure 8). The Australian Government's share of total health expenditure was its lowest over the decade to 2024–25, while the state and territory governments was its highest."
Section: Figure 8: Proportion of total health spending by source of funds, current prices, 2014–15 to 2024–25 · Source: Health expenditure Australia 2024–25

Why it matters

The Australian Government's funding share falling to a decade low of 39.2% while states reached 30.8% may signal a structural shift in who bears healthcare costs. If the Commonwealth continues to pull back, cost pressures could increasingly fall on states, individuals and private insurers, potentially affecting member out-of-pocket costs and the attractiveness of private cover. Rated medium because the trend is sustained over a decade and the direction is clear, but the direct mechanism through which this affects Medibank's claims or benefits requires further investigation.

MEDIUM PRIORITY *Insight 25* · *AIHW* · *Clinical*

First Nations Australians incur hospitalisation expenditure 1.6 times higher per person than non-Indigenous Australians ($5,683 vs $3,508), with the gap widest among working-age adults aged 35–54, where First Nations expenditure is 3.2 times higher per person.

Evidence

"Across all disease groups, expenditure on hospitalisations was $5,683 per First Nations person compared with $3,508 per non-Indigenous person. This means First Nations expenditure was 1.6 times as high in relative terms, and $2,175 higher per person in absolute terms... Age-specific results suggest particularly large expenditure gaps among working-age adults, especially those aged 25–54 years. For age groups 35–44 and 45–54, First Nations expenditure per person was 3.2 times that for non-Indigenous people."
Section: Figure 26: Spending on hospitalisations by burden of disease(a) or injury group by Indigenous status, 2024–25 · Source: Health system spending on disease and injury in Australia 2024–25

Caveat

This section only includes expenditure for hospitalisations and does not capture spending on primary health care or referred medical services.

Why it matters

The 1.6-times-higher hospitalisation expenditure per First Nations person, rising to 3.2 times among 35–54-year-olds, signals a substantial health-outcome and service-demand gap that could affect Medibank members in this population. The gap points to higher disease burden and potentially more complex or acute hospital episodes. It is rated medium because the evidence is hospitalisation-only and the proportion of First Nations people holding private health insurance is relatively small, limiting the direct claims pathway. Still useful for Medibank's health equity and member-outcome monitoring.

MEDIUM PRIORITY *Insight 26* · *AIHW* · *Clinical*

Cancer spending for males now peaks in the 75–79 age group for the first time in 2024–25, having previously peaked at 70–74, indicating that cancer treatment is being delivered to older male patients than in prior years.

Evidence

"From 2017–18 to 2023–24, spending on cancers peaked in the age group 70–74 years for both males and females. In 2024–25, for the first time, spending on cancers for males is now peaking in the 75–79 years age group."
Section: Figure 17: Spending on disease groups(a), by age group and sex, 2013–14 to 2024–25 ($ million) · Source: Health system spending on disease and injury in Australia 2024–25

Why it matters

The shift of peak male cancer spending from the 70–74 to the 75–79 age group may affect Medibank's claims profile, as older members tend to have longer hospital stays, more comorbidities and higher per-episode costs. This could signal that more intensive cancer treatment is being offered to older men, or that the insured male population is ageing into higher-cost cancer care. It is rated medium because the shift is a single-year observation and the magnitude is a one-age-band move, so the trend is not yet firmly established. Worth tracking in future years alongside Medibank's own age-specific cancer claims data.

MEDIUM PRIORITY *Insight 27* · *AIHW* · *Clinical*

For cancer, declining disease burden reduced spending for both sexes, but the reduction was larger for males ($1.27 billion, 34.4%) than females ($0.71 billion, 21.8%), while increased service use per DALY added more for males (45.2%) than females (33.1%).

Evidence

"For cancer and other neoplasms, lower disease burden was associated with a reduction in spending for both males and females, with a larger reduction for males ($1.27 billion, or 34.4%) in comparison to females ($0.71 billion, or 21.8%). Similarly, increased services per DALY was associated with an increase in spending for both groups, again with a larger effect for males (45.2%) than females (33.1%)."
Section: Decomposition of disease expenditure by burden of disease or injury group, age and sex · Source: Health system spending on disease and injury in Australia 2024–25: Growth decomposition of health system spending on disease and injury in Australia

Caveat

Results represent estimated contributions under the decomposition model and should not be interpreted as independent causal effects.

Why it matters

The sex-based difference in cancer spending drivers — males seeing larger burden reductions but also larger service-intensity increases — could affect how Medibank plans cancer-related benefits and hospital contracts. More services per unit of illness for males may reflect expanded screening, treatment regimens or survivorship care that flows into private oncology claims. It is rated medium rather than high because the finding adds useful context on cancer cost dynamics but does not identify a specific service shift or cost pressure point as directly as the condition-level insights above.

MEDIUM PRIORITY *Insight 28* · *AIHW* · *Clinical*

Changes in disease burden had a negligible or negative effect on PBS spending across the decade, in contrast to all other service areas where rising burden contributed positively — suggesting PBS expenditure growth is driven by factors other than population illness levels.

Evidence

"Changes in disease burden were generally associated with a smaller effect and increased spending across all areas, with the exclusion of the PBS."
Section: Decomposition of disease expenditure by detailed area of expenditure · Source: Health system spending on disease and injury in Australia 2024–25: Growth decomposition of health system spending on disease and injury in Australia

Caveat

Results represent estimated contributions under the decomposition model and should not be interpreted as independent causal effects.

Why it matters

PBS spending growth being disconnected from disease burden levels suggests that new medicines, pricing changes or expanded indications — rather than sicker populations — are driving pharmaceutical costs. This could affect Medibank's ancillary and hospital-related pharmaceutical costs if the same dynamic applies to medicines used in private settings. It is rated medium because the finding identifies a structural cost driver worth monitoring, but the evidence is high-level and does not specify which therapeutic classes or conditions are responsible. Worth examining whether Medibank's pharmaceutical-related claims show cost growth independent of member illness patterns.

MEDIUM PRIORITY *Insight 29* · *AIHW* · *Clinical*

Chronic lower respiratory disease is recorded in 19% of people with schizophrenia or affective psychosis and 25% of those with combined mental health conditions, roughly double the 9.9% rate in people without mental health conditions.

Evidence

"The proportion of CLRD was almost double among people with schizophrenia and/or affective psychosis (19%) compared with people without these conditions (9.9%)."
Section: Figure 5: Types of physical health conditions among people with a recorded mental health condition in general practice in WA, 2023–24 · Source: General practice care for people with a mental health condition

Caveat

CLRD in this study includes asthma, COPD, and chronic bronchitis combined; the national comparator estimate of 12% covers all ages, while this study covers adults 18 and over only.

Why it matters

Chronic lower respiratory disease at roughly double the rate in people with mental health conditions (19–25% vs 9.9%) could mean higher respiratory-related GP visits, medication costs and hospital admissions among affected Medibank members. The connection runs through higher smoking rates and reduced preventive care access in this group. It is rated medium rather than high because the CLRD category combines asthma, COPD and chronic bronchitis, making the specific clinical and cost pathway less precise, and the affected population is relatively small. Medibank could examine respiratory claims among members with co-occurring mental health diagnoses.

MEDIUM PRIORITY *Insight 30* · *AIHW* · *Clinical*

Queensland has the highest powered scooter hospitalisation rate in Australia at 27 per 100,000 in 2024–25 — more than three times the rate in New South Wales (7.8 per 100,000), directly tracking the permissiveness of each state's e-scooter laws.

Evidence

"Queensland had the highest rate in 2024–25 (27 per 100,000 population). Queensland was an early adopter of e-scooter use and has allowed relatively broad public access, including private use in public areas... New South Wales recorded the lowest hospitalisation rate in 2024–25 (7.8 per 100,000 population). This may reflect ongoing restrictions on private e-scooter use on public land and access largely limited to approved shared scheme trial areas."
Section: State and territory differences · Source: E-scooter and e-bike injuries and deaths in Australia: national trends and data limitations

Why it matters

The more than threefold difference in hospitalisation rates between Queensland and New South Wales suggests Medibank's hospital claims burden from these injuries could vary substantially by state, tracking how permissive local e-scooter laws are. This may be useful for state-level claims forecasting and product pricing. It is rated medium rather than high because the insight is primarily a cross-jurisdictional comparison rather than a new trend, and the coding caveat still applies. Comparing Medibank's own state-level trauma claims patterns against these regulatory differences could be informative.

MEDIUM PRIORITY *Insight 31* · *AIHW* · *Clinical*

New ICD-10-AM placeholder codes for e-scooters, e-bikes and other e-mobility devices took effect from 1 July 2026, meaning hospitalisation data from 2026–27 onwards will for the first time separately identify these injuries rather than grouping them with mobility scooters and motorcycles.

Evidence

"As of 1 July 2026, placeholder codes for e-scooters, e-bikes and other e-mobility devices come into effect for ICD-10-AM Thirteenth Edition (IHACPA 2026). This will allow hospitalisations involving e-scooters to be distinguished from mobility scooters and other pedestrian conveyances, e-bikes to be distinguished from motorcycles, and other powered pedestrian conveyances such as powered skateboards, one-wheeled boards and e-boards to be distinguished from their non-powered counterparts."
Section: Understanding e-mobility injury trends and data limitations · Source: E-scooter and e-bike injuries and deaths in Australia: national trends and data limitations

Caveat

The new codes are described as 'placeholder' codes; the report does not specify the level of clinical detail they will capture beyond device type.

Why it matters

The new ICD-10-AM placeholder codes from 1 July 2026 will allow Medibank to separately identify e-scooter and e-bike hospital claims from 2026–27 onwards, removing the current ambiguity with mobility scooters and motorcycles. This is an operational data-quality improvement rather than a clinical finding, but it enables more precise claims analysis and injury monitoring. It is rated medium because it creates a concrete near-term opportunity to build e-mobility injury tracking into Medibank's analytics once coded data begins flowing.

MEDIUM PRIORITY *Insight 32* · *AIHW* · *Clinical*

Pedestrian injuries from unspecified transport accidents nearly tripled from 0.9 to 2.4 per 100,000 between 2022–23 and 2024–25, suggesting a growing number of third-party e-mobility injuries are being miscoded and the true pedestrian injury burden is understated.

Evidence

"The category Pedestrian injured in other and unspecified transport accidents has seen an increase in hospitalisations in recent years. The rate rose from 0.9 per 100,000 in 2022–23 to 1.9 per 100,000 in 2023–24 and further to 2.4 per 100,000 in 2024–25. This classification may be increasingly used to record pedestrian injuries resulting from collisions with e-scooters or e-bikes."
Section: Trends in hospitalisations for transport groups that include e-scooters and e-bikes · Source: E-scooter and e-bike injuries and deaths in Australia: national trends and data limitations

Caveat

The report explicitly states this is a possible interpretation; the coding ambiguity means the true cause cannot be confirmed from current data.

Why it matters

The near-tripling of pedestrian injuries coded as unspecified transport accidents may indicate that third-party e-mobility injuries affecting Medibank members are being undercounted in current claims data, meaning the true hospital burden from e-scooter and e-bike collisions could be larger than existing coding suggests. This matters for understanding the full scope of e-mobility-related claims. It is rated medium rather than high because the report explicitly notes this is a possible interpretation and the coding ambiguity cannot be resolved with current data. Worth revisiting once the new ICD-10-AM codes enable clearer identification from 2026–27.

WATCHLIST *Insight 33* · *AIHW* · *Clinical*

The National Lung Cancer Screening Program launched in July 2025 is not yet costed in this report; its expenditure will appear in future editions, but the program targets high-risk individuals using low-dose CT and is expected to add a new screening cost line alongside the existing $508.7 million spent on breast, cervical and bowel screening.

Evidence

"The National Lung Cancer Screening Program was implemented in July 2025. The Program uses low-dose computed tomography (low-dose CT) scans to look for lung cancer in high-risk people without any symptoms. It aims to find lung cancer early and save lives. Expenditure on The National Lung Cancer Screening Program will be included in future editions of this report."
Section: Spending on cancer screening · Source: Health system spending on disease and injury in Australia 2024–25

Caveat

Expenditure on the National Lung Cancer Screening Program is not yet included in this report and will appear in future editions.

Why it matters

The new National Lung Cancer Screening Program using low-dose CT could generate downstream private-sector activity if screen-detected cancers lead to earlier surgical and oncology treatment in private hospitals for insured members. However, the program's costs are not yet quantified in this report and its impact on private health claims is speculative at this stage. It is rated watchlist because the program is confirmed but expenditure data and utilisation patterns are not yet available. Worth monitoring future AIHW editions for cost data and tracking any uptick in early-stage lung cancer treatment claims among Medibank members.

WATCHLIST *Insight 34* · *AIHW* · *Clinical*

Hallucinogen use among people in their 20s more than doubled between 2016 and 2022–23, from 3.1% to 6.8%, while ketamine use reached 4.2% in this age group — both trends continuing upward.

Evidence

"Hallucinogen use among people in their 20s has increased steadily since 2001. Between 2016 and 2022–2023, hallucinogen use in this age group more than doubled – from 3.1% in 2016 to 6.8% in 2022–2023 (AIHW 2024b). Use of hallucinogens and ketamine in the last 12 months was most common among people aged 20–29 (6.8% and 4.2% respectively)."
Section: Figure 4: Proportion of people aged 20–29 who recently used ecstasy, cocaine or hallucinogens, 2001 to 2022–2023 · Source: Illicit drug use

Why it matters

Rising hallucinogen and ketamine use among people in their 20s could eventually increase emergency presentations and mental-health service demand among younger Medibank members. The doubling of hallucinogen use from 3.1% to 6.8% over six years is a clear upward trend in a moderate-sized population. However, the supplied evidence describes prevalence of use rather than current hospitalisations, treatment episodes or healthcare costs, so the pathway to Medibank claims remains indirect. This is worth monitoring for any future emergence of related acute-care or psychiatric admissions in Medibank data.

WATCHLIST *Insight 35* · *AIHW* · *Clinical*

The ACT shows that tighter regulation can reverse hospitalisation trends: rates rose sharply after e-scooter legalisation and shared scheme expansion from 2019–20, then declined after safety rule changes — a pattern not seen in Queensland or Western Australia.

Evidence

"The Australian Capital Territory recorded a sharp increase after personal e-scooters were legalised from late 2019, plus a rapid expansion of shared schemes from 2020. Rates later declined after changes to safety rules and scheme arrangements."
Section: State and territory differences · Source: E-scooter and e-bike injuries and deaths in Australia: national trends and data limitations

Caveat

ACT numbers are relatively small and the report notes the Northern Territory trend should be interpreted cautiously due to small numbers; the same caution may apply to ACT rate movements.

Why it matters

The ACT experience suggests that tighter safety regulation can reduce e-scooter hospitalisations, which could in turn reduce associated hospital claims for Medibank members in affected jurisdictions. However, the ACT population is small, the report cautions about interpreting small-number trends, and the finding is more relevant to public policy than to direct Medibank action. It is placed on the watchlist because similar regulatory changes in larger states could materially shift claims patterns, making it worth monitoring without requiring active investigation now.

Pharmaceutical Insights

HIGH PRIORITY *Insight 36* · *AIHW* · *Pharmaceutical*

PBS spending on cancer medicines has been more than double that of the next highest disease group for five consecutive years (since 2019–20), reflecting the sustained listing of high-cost oncology drugs on the PBS.

Evidence

"For the past 5 years (since 2019–20), PBS spending on cancers and other neoplasms was more than double the spending of the next highest disease group."
Section: Figure 18: Spending on disease groups(a), by area of expenditure, current and constant prices, 2013–14 to 2024–25 ($ million) · Source: Health system spending on disease and injury in Australia 2024–25

Why it matters

PBS cancer medicine spending being more than double the next disease group for five consecutive years signals sustained high-cost oncology drug utilisation that may flow through to Medibank via pharmaceutical benefits, gap payments and associated treatment costs for members undergoing cancer care. The persistent dominance reflects ongoing listings of expensive targeted and immunotherapy agents. It is rated high because the trend is strong, the cost magnitude is large, and cancer medicines increasingly accompany private hospital admissions and day procedures. Worth monitoring whether Medibank's pharmaceutical-related cancer costs are growing at a similar pace.

HIGH PRIORITY *Insight 37* · *AIHW* · *Pharmaceutical*

Polypharmacy affects 1.9 million Australians — 7% of the population — and is concentrated heavily in older age groups, with 40% of people aged 75 and over regularly dispensed five or more medicines in 2025.

Evidence

"In 2025, around 1.9 million people in Australia (7.0% of the Australian population) experienced polypharmacy, having been regularly dispensed 5 or more PBS/RPBS medicines during the year... Around 910,000 people aged 75 years and over were dispensed 5 or more PBS/RPBS medicines during the year, equivalent to 40% of the population aged 75 years and over. This compares to 20% of the population aged 65–74 years who were dispensed 5 or more medicines during the year, and 9.5% of the population aged 55–64 years."
Section: Polypharmacy by age and sex · Source: Polypharmacy in Australia in 2025

Caveat

Medicines supplied through private prescriptions are not included, so polypharmacy rates may underestimate the true extent of polypharmacy in Australia.

Why it matters

This may affect Medibank members' medicine-related claims, medication management services and health outcomes, particularly among older members where 40% of those aged 75 and over are on five or more medicines. The sheer scale — 1.9 million Australians — means a substantial share of Medibank's membership, especially in older cohorts, could be affected by polypharmacy-related adverse events, hospitalisations and medicine reviews. It is rated high because the current burden itself is material and identifies a concrete member-safety and service-planning issue. Worth checking whether Medibank claims data show corresponding patterns in medication reviews, adverse drug events or hospital admissions among members on multiple medicines.

HIGH PRIORITY *Insight 38* · *AIHW* · *Pharmaceutical*

Around 219,000 Australians — 1% of the population and 11% of those already on polypharmacy — were regularly dispensed ten or more different medicines in 2025, a level known as hyperpolypharmacy.

Evidence

"Around 11% of people experiencing polypharmacy (219,000, representing 1% of the Australian population) were experiencing hyperpolypharmacy (where people were regularly dispensed 10 or more medicines, Davison et al. 2026)."
Section: Polypharmacy by number of medicines dispensed · Source: Polypharmacy in Australia in 2025

Caveat

Medicines supplied through private prescriptions are not included, so the true number may be higher.

Why it matters

Hyperpolypharmacy — ten or more regular medicines — may identify a subset of Medibank members at significantly elevated risk of adverse drug interactions, hospitalisations and avoidable harm. The 219,000 Australians in this group are likely high healthcare utilisers whose outcomes could be improved through targeted medication review or care coordination. It is rated high because this level of medicine use is a well-recognised clinical safety concern and the population is identifiable, making it directly investigable. Medibank could examine whether its own members in this category show higher hospital admission or adverse event rates.

HIGH PRIORITY *Insight 39* · *AIHW* · *Pharmaceutical*

GLP-1 analogue prescriptions nearly doubled in three years, rising 87% from 1.7 million in 2021–22 to 3.2 million in 2024–25, making semaglutide (Ozempic) the seventh most costly PBS-subsidised medicine.

Evidence

"The largest increase of 87% was seen for GLP-1 analogues from 1.7 million in 2021–22 to 3.2 million in 2024–25 (Figure 1; select 'Diabetes medicine type' view). Additionally, Semaglutide (Ozempic), a glucagon-like peptide-1 analogue for treatment of type 2 diabetes, was the seventh most costly PBS subsidised medicine in 2024–25 (Department of Health, Disability and Ageing 2025)."
Section: Medicine use · Source: Diabetes: Australian facts

Why it matters

This may directly affect Medibank's pharmaceutical benefits costs and member out-of-pocket expenses for type 2 diabetes treatment. An 87% increase in GLP-1 analogue prescriptions over three years, reaching 3.2 million scripts and making semaglutide the seventh most costly PBS medicine, signals rapidly growing demand that could flow through to private health claims and ancillary costs. It is rated high because the trend is sustained, the cost and volume signals are substantial, and the medicine class is already among the most expensive nationally. Worth checking whether Medibank members' GLP-1 analogue dispensing and associated diabetes management claims mirror this national growth trajectory.

HIGH PRIORITY *Insight 40* · *AIHW* · *Pharmaceutical*

Glycaemic control among people attending diabetes services remains poor: median HbA1c was 7.9% overall in 2024, above the ≤7% target, and only 30% of patients met the target—a signal that current treatment is not achieving optimal outcomes for most patients.

Evidence

"Of the nearly 3,000 people aged 18 and over with diabetes who had an HbA1c measurement recorded in 2024, the median HbA1c was 7.9% (above the ≤7% target). Median HbA1c was 7.7% among people with type 1 diabetes and 8.0% in people with type 2 diabetes. Among people with diabetes who had an HbA1c measurement recorded, 30% had the target HbA1c of ≤7%, the optimum goal for the management of diabetes, with 31% and 29% of those with type 1 and type 2 diabetes meeting the target HbA1c, respectively."
Section: Pathology · Source: Diabetes: Australian facts

Caveat

Data are from the Australian Diabetes Clinical Quality Registry, which covers only people attending participating diabetes services; this cohort may not be representative of all Australians with diabetes.

Why it matters

This may affect Medibank members with diabetes through higher rates of complications, hospitalisations and ongoing treatment costs, since only 30% of patients are meeting the recommended HbA1c target. Poor glycaemic control is a well-established driver of diabetes-related complications such as kidney disease, eye disease and cardiovascular events, all of which generate substantial hospital and specialist claims. It is rated high because the current gap between actual and target glycaemic control is large and points to a material unmet-need issue affecting a common chronic condition. Worth investigating whether Medibank's diabetes members show similar patterns of suboptimal control and higher complication-related claims.

MEDIUM PRIORITY *Insight 41* · *AIHW* · *Pharmaceutical*

In younger people experiencing polypharmacy, mental health medicines — antidepressants, antipsychotics and psychostimulants — dominate the medicine profile, while chronic disease medicines (diabetes, cardiovascular, dyslipidaemia) dominate from middle age onwards.

Evidence

"In younger age groups, polypharmacy was characterised by medicines for mental health and neurological conditions, particularly antidepressants, antipsychotics and psychostimulants. This is reflected in the fact that long-term mental health conditions that are most commonly diagnosed in childhood (such as ADHD and autism) tend to feature in multimorbidity among younger age groups (AIHW 2025). In older adults, the pattern shifted towards medicines used to manage chronic physical health conditions, including diabetes, cardiovascular disease, dyslipidaemia and gastro-oesophageal reflux disease (GORD). Antidepressants remained a common feature across most age groups, including in older adults."

Section: Polypharmacy by number of medicines dispensed · Source: Polypharmacy in Australia in 2025

Why it matters

This may help Medibank understand which medicine classes drive polypharmacy across different member age groups, informing targeted medication review or chronic disease management programs. The dominance of mental health medicines in younger cohorts and chronic disease medicines in older cohorts suggests different intervention strategies may be needed. It is rated medium because it provides useful context for service planning and member segmentation but does not by itself quantify a specific utilisation change or cost impact. Medibank could compare its dispensing profiles to see whether the same age-specific patterns hold among its members.

MEDIUM PRIORITY *Insight 42* · *AIHW* · *Pharmaceutical*

Among people aged 65–74, polypharmacy is more common in men than women (22% versus 18%), reversing the pattern seen in younger adults where women have higher rates — consistent with older men carrying a higher burden of multiple chronic conditions.

Evidence

"The largest difference was observed among people aged 65-74 years (22% of the male population compared to 18% of the female population, Figure 1). This is consistent with findings that older males are more likely to be living with 2 or more chronic conditions at the same time (multimorbidity) compared to older females (AIHW 2025) and therefore require more medication to manage these conditions."

Section: Polypharmacy by age and sex · Source: Polypharmacy in Australia in 2025

Why it matters

This may help Medibank tailor chronic disease and medication management programs for older male members, who appear to carry a higher multimorbidity and polypharmacy burden in the 65–74 age group. The sex difference — 22% of men versus 18% of women — points to a specific demographic segment where intervention could be concentrated. It is rated medium because the finding is useful for member segmentation and program design but the absolute difference is modest and no cost or utilisation change is quantified. Medibank could check whether its own older male members show similarly elevated polypharmacy rates.

MEDIUM PRIORITY *Insight 43* · *AIHW* · *Pharmaceutical*

Around 8 million Australians (29% of the population) were regularly taking one to four medicines in 2025, with those on three or four medicines identified as most likely to transition into polypharmacy over time.

Evidence

"In 2025, around 8 million people in Australia (29% of the population) were taking 1–4 medicines. Across all age groups, people taking 1–4 medicines outnumbered people experiencing polypharmacy (Figure 1)... In particular, people regularly dispensed 3 or 4 medicines may be more likely to transition to polypharmacy over time."
Section: Comparison to people taking 1 to 4 medicines · Source: Polypharmacy in Australia in 2025

Caveat

Longitudinal analysis would be required to determine whether people taking 1–4 medicines transition to polypharmacy over time; this interpretation should be treated cautiously.

Why it matters

This may help Medibank identify members at risk of transitioning into polypharmacy, particularly the subset currently on three or four regular medicines. Early intervention for these members — through medication reviews or chronic disease support — could potentially prevent avoidable polypharmacy and its associated health risks. It is rated medium rather than high because the transition pathway is hypothesised rather than observed, and longitudinal evidence is not yet available. Medibank could monitor members on three to four medicines over time to see whether they progress to five or more and what triggers that shift.

INFORMATIONAL *Insight 44* · *AIHW* · *Pharmaceutical*

Australian Government PBS subsidies totalled $18.6 billion in 2024–25, while private health insurers spent $22.1 billion on all health services combined — highlighting the PBS as a cost centre of comparable scale to the entire private insurance benefit pool.

Evidence

"During the same year, the Australian Government funded $18.6 billion as subsidies for PBS and $0.4 billion for RPBS pharmaceuticals. During 2024–25, providers of private health insurance financed $22.1 billion (7.6%) of total health spending."
Section: MBS, PBS and RPBS government benefits paid in 2024–25 · Source: Health expenditure Australia 2024–25

Why it matters

The comparison of $18.6 billion in PBS subsidies against $22.1 billion in total private insurance spending provides useful context on the relative scale of pharmaceutical and private insurance funding pools. However, the insight does not identify a change, trend or specific issue that Medibank could act on — it is primarily a static size comparison. Rated informational because while the scale context is interesting, it does not point to a concrete claims, benefits or member-outcome question for Medibank to investigate.

Prosthesis & Medical Technology Insights

No insights identified for this period.